TapestryHealth™

January 27, 2026

Jim O'Neill
Deputy Secretary
U.S. Department of Health and Human Services
Office of the Deputy Secretary
200 Independence Avenue, S.W.
Washington, D.C. 20201

Re: TapestryHealth's Response to RIN 0955-AA13 – Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Deputy Secretary O'Neill:

TapestryHealth is pleased to provide the following information to the U.S. Department of Health and Human Services (HHS) in response to its Request for Information regarding the adoption and use of artificial intelligence (AI) as part of clinical care (RIN 0955-AA13). *See* 90 Fed. Reg. 60108 (Dec. 23, 2024). TapestryHealth is a specialized medical practice serving the Post-Acute and Long-Term Care (LTC) sector. Our responses below reflect the unique operational realities of Skilled Nursing Facilities (SNFs) and Assisted Living Communities. While some principles may apply broadly, our recommendations are specifically designed to address the high-acuity, low-staffing, and multi-chronic nature of this vulnerable patient population, which often differs significantly from ambulatory or acute hospital settings.

TapestryHealth currently monitors over 80,000 patients in elder care communities, capturing vital cardiopulmonary data every second. We are writing to address a critical disconnect: While AI technology has matured to the point where it can predict hospitalizations 5 days in advance, the current payment models actively penalize providers for using it. We note that many of the suggestions below specifically address reimbursement matters centered on the Centers for Medicare & Medicaid Services (CMS).

I. The Core Barrier: The "Efficiency Penalty"

The primary barrier to AI adoption is not technological—it is economic. The current "Labor-Based" pricing model (RVUs) is incompatible with AI-native care.

- • The Efficiency Penalty: Current CPT codes equate "value" with "time spent." If an AI tool allows a clinician to review a patient's status in 2 minutes instead of 20, or allows the same 20-minute increment of clinical time to be more comprehensive and effective, the provider is financially penalized. This creates an incentive to maintain slower, manual workflows.
- • Another impact to efficiency and effectiveness is that providers cannot concurrently bill for RPM (Remote Physiologic Monitoring) and RTM (Remote Therapeutic Monitoring). This forces

providers to choose between the services, creating "data silos" where AI models are blinded to half the patient's context.

II. Proposed Solution: A Tiered Reimbursement Model

We propose a Tiered Complexity Model for AI reimbursement, as detailed in our CY 2026 PFS comments:

- Tier 1 (Administrative AI): Simple rule-based alerts. Reimbursement covers basic software costs.
- Tier 2 (Assistive AI): Pattern recognition on single data streams. Reimbursement covers software + moderate clinician and clinical staff review.
- Tier 3 (Predictive/Multimodal AI): High-complexity modeling using continuous, passive data. Reimbursement covers infrastructure + high-level cognitive interpretation by clinicians and clinical staff.

III. Safety & Oversight: The Indispensable "Human-in-the-Loop"

AI should not replace the clinician; it should elevate them.

- The "Signal-to-Noise" Problem: Pure software solutions often generate hundreds of false alarms. In the SNF setting, "alert fatigue" is a major safety risk.
- Recommendation: HHS and CMS should require—and reimburse—a "Clinical Verification" component. We propose a new code for "AI-Guided Clinical Interpretation" that pays for the cognitive labor of verifying AI insights.

IV. Real-World Efficacy: Contactless Radar Monitoring

- Early Warning: Contactless monitoring provided an average predictive lead time of 4.99 to 5.65 days prior to a hospitalization event.
- Outcome Improvement: Facilities utilizing this "AI + Clinician" model saw a 43% reduction in ED visits and a 45% reduction in inpatient admissions.

V. Use of AI Technology for Basic Analytics

AI significantly enhances the ability to generate meaningful and actionable insights from the vast, disparate, and often unstructured data residing within long-term care electronic medical record systems. While remote vitals monitoring is one application, broader use of AI analytics remains underutilized due to current financial models that do not incentivize these technological advancements.

By applying AI to a patient's prescribed drug history in combination with other medical information, clinicians can develop actionable plans aimed at improving overall health outcomes and reducing avoidable hospital admissions. For example, an AI model can flag a heart failure patient on high-dose diuretics whose respiratory rate has subtly increased over 48 hours – a pattern often missed by routine checks but highly predictive of an impending fluid overload crisis. This approach leverages AI's capability to analyze complex data sets and identify patterns that may otherwise go unnoticed. Additionally, AI can help identify specific opportunities for behavioral health integration within the long-term care setting. For example, AI can analyze movement patterns to detect when a typically active resident begins spending prolonged periods alone in their room, prompting a depression screening before social withdrawal deepens. Through its advanced analytics, AI supports the recognition of behavioral health needs and facilitates the development of intervention plans tailored to individual patients. Additionally, AI can help identify specific opportunities for behavioral health integration within the long-term care setting. Through its advanced analytics, AI supports the recognition of behavioral health needs and facilitates the development of intervention plans tailored to individual patients.

VI. Responses to Specific RFI Questions

Q1: Biggest barriers to private sector innovation & adoption?

Response: The "Efficiency Penalty" in Reimbursement.

In the low-margin SNF sector, adoption is driven by ROI. The biggest barrier is that current reimbursement (RVUs) pays for time spent, not outcomes achieved. AI reduces time spent, effectively lowering revenue for efficient providers. Until payment models reward "Capacity" (patients managed safely) rather than "Minutes" (time spent documenting), innovation will remain a financial liability for LTC providers.

Q2: Regulatory/payment changes to prioritize?

Response: Unblock Data Silos & Value Cognitive Interpretation.

1. Unblock Concurrent Billing (Amend 42 C.F.R. § 410.78): CMS must permit concurrent billing of RPM (99454) and RTM (98975) when distinct hardware/data streams are used. For complex SNF patients, AI needs both physiological and therapeutic data to be safe and effective. In addition to removing the subregulatory guidance that prohibits concurrent billing of 99454 and 98975, CMS should amend 42 C.F.R. § 410.78 to explicitly permit concurrent billing for services that utilize distinct hardware/data streams.

2. Create "AI-Guided Interpretation" Codes (Revisit 42 C.F.R. § 414.20): Introduce add-on codes that specifically reimburse the "Human-in-the-Loop" verification step, ensuring safety without penalizing the provider for the cost of the software.

Q5: How can HHS support private sector (accreditation)?

Response: Standardize "Contactless & Ambient" Monitoring.

HHS should establish an accreditation class for "Ambient Vital Sign Monitoring" (e.g., radar/LiDAR). In the SNF sector, wearables have high failure rates due to dementia and skin fragility. Validating "Contactless" as a standard of care is crucial for equitable access in this population.

Q7: Decision makers & administrative hurdles?

Response: The "Capital vs. Operational" Mismatch for Facility Administrators.

The SNF Administrator is the key decision-maker but operates on a fixed per-diem. The hurdle is that AI requires upfront Capital Expense (sensors), but savings (reduced hospitalizations) accrue to the Payer (Medicare), not the Facility. Payment models must share these savings with the facility to incentivize infrastructure investment.

Q9: Patient/Caregiver challenges & concerns?

Response: The Tension Between "Safety" and "Surveillance".

Families want the safety of continuous monitoring ("never alone"), but residents fear the indignity of cameras ("Big Brother"). This specifically validates the need for Privacy-Preserving AI (Contactless Radar) which provides the safety of an ICU monitor without the invasion of a camera.

VII. Conclusion

The "AI Doctor" will not work for free—and neither can the human clinicians who supervise them. By modernizing payment rails to reward preventive efficiency and verified oversight, HHS and CMS can solve the staffing crisis and dramatically improve the quality of life for our nation's seniors.

TapestryHealth thanks the Deputy Secretary for his attention to this matter. We would be happy to provide any additional information or materials that you may need in assessing how to integrate AI into clinical care services.

Sincerely,

Mordy Eisenberg, NHA
Chief Growth Officer
TapestryHealth